This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Desogestrel 75 microgram tablets and the administration of Medroxyprogesterone acetate 150mg/ml injection (Depo-Provera) for Postnatal Contraception

Patient Group Direction, version 008, issued 30 September 2026. Supersedes Version 007, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for postnatal contraception: CoSRH (formerly FSRH) clinical guideline Contraception After Pregnancy (2017, amended 2020), UKMEC 2025, and the SmPCs of the products supplied. NICE CKS 'Contraception - assessment' summarises the same sources.

Overview

- Fertility can return as early as 21 days post-delivery.
- Contraception advice should be provided during postnatal care.
- Progesterone-only methods are safe during breastfeeding.

Assessment

- Delivery date
- Breastfeeding status
- Medical history
- VTE risk
- Previous contraceptive use

- Postnatal complications

Management

- Progesterone-only pill (POP) can start any time postpartum (day 1 or later).
- Depo-Provera from 6 weeks postpartum if breastfeeding; from 21 days if not breastfeeding and no additional VTE risk factor.
- UKMEC 2025 criteria apply. For the progestogen-only pill the category 4 condition is current breast cancer; the category 3 conditions are past breast cancer, ischaemic heart disease or stroke (continuation), severe cirrhosis, liver tumours (hepatocellular adenoma or carcinoma) and enzyme-inducing medicines. These are named exclusions in the desogestrel arm, which also excludes SLE with antiphospholipid antibodies.

Red flags - seek urgent medical advice

- DVT/PE symptoms
- Breast cancer (current)
- Severe liver disease
- Unexplained vaginal bleeding

Patient Group Direction

for the supply of

Desogestrel 75mcg tablets (Cerazette/Cerelle)

for the treatment of

Postnatal Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed

	equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Postnatal contraception for women aged 16+
Inclusion criteria	Postnatal women aged 16 years and over Any time postpartum. Before day 21 no additional contraception is needed. From day 21, pregnancy must be reasonably excluded (no unprotected intercourse since day 21, or a negative test 21 days after the last episode) and 2 days of extra precautions advised (FSRH; the SmPC gives 7 days). Informed consent obtained UKMEC 2025 category 1 or 2 for the progestogen-only pill met: none of the category 3 or 4 conditions named under exclusion criteria is present
Exclusion criteria	Known or suspected pregnancy Active thromboembolic disorder Sex hormone-dependent malignancy (current or suspected breast cancer) Severe hepatic impairment or liver tumours Undiagnosed vaginal bleeding Hypersensitivity to desogestrel or any excipients UKMEC 2025 category 3 or 4 for the progestogen-only pill. Any of the following excludes; refer to the GP or sexual health service:

	• current breast cancer (category 4) • past breast cancer, whatever the interval since treatment (category 3) • ischaemic heart disease, current or past, or a history of stroke or transient ischaemic attack (category 3 for continuation; excluded for starts and for continuation under this PGD) • severe (decompensated) cirrhosis (category 3) • hepatocellular adenoma or hepatocellular carcinoma (category 3) • an enzyme-inducing medicine taken now or within the last 28 days: rifampicin, rifabutin, carbamazepine, oxcarbazepine, eslicarbazepine, phenytoin, phenobarbital, primidone, topiramate, St John's wort, efavirenz, nevirapine or a ritonavir-boosted protease inhibitor (category 3) • systemic lupus erythematosus with positive or unknown antiphospholipid antibodies
Cautions	Functional ovarian cysts Diabetes Hypertension Migraine Depression
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Desogestrel 75 micrograms tablets
Legal category	POM
Storage	Store below 30°C
Route/method of administration	Oral, one tablet daily
Dose and frequency	One tablet daily at the same time each day, continuously (no pill-free break). 12-hour window for missed pills. Start any time postpartum. Started up to and including day 21: no additional contraception needed. Started after day 21: exclude pregnancy first and use a barrier method for 2 days (FSRH; the SmPC states 7 days).
Quantity to be administered	Up to 3 months' supply (3 x 28 tablets = 84 tablets)

and/or supplied	
Maximum or minimum treatment period	Continuous until change of contraception or pregnancy
Adverse effects	Common: Irregular bleeding or spotting, headache, mood changes, breast tenderness, nausea, acne, decreased libido, weight gain Uncommon: Alopecia, ovarian cysts, fatigue

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Discuss breastfeeding safety and the delayed return of fertility with Depo-Provera.
Follow-up advice to be given to patient or carer	Irregular bleeding is common with this method, particularly in the first few months. This method is safe during breastfeeding. Take the tablet at the same time each day. If you are more than 12 hours late, take it as soon as you remember and use condoms for the next 2 days.

	Seek medical advice if symptoms worsen or any adverse effects occur. Seek immediate medical attention if symptoms of DVT/PE develop (calf pain, swelling, breathlessness). Report any unexpected vaginal bleeding. Discuss contraceptive options if considering longer-term contraception.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- College of Sexual and Reproductive Healthcare (formerly FSRH), Contraception After Pregnancy, January 2017 (amended October 2020); UKMEC 2025; Summaries of Product Characteristics for the products supplied, current versions
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the administration of

Medroxyprogesterone acetate 150mg/ml injection (Depo-Provera)

for the treatment of

Postnatal Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Postnatal injectable contraception for women aged 18+
Inclusion criteria	Postnatal women aged 18 years and over From 6 weeks postpartum if breastfeeding. From 21 days postpartum if NOT breastfeeding and there is no additional VTE risk factor (previous VTE, thrombophilia, immobility, BMI 30 or over, caesarean delivery, postpartum haemorrhage, pre-eclampsia, smoking). Otherwise refer. Informed consent obtained UKMEC 1 or 2 criteria met
Exclusion criteria	Known or suspected pregnancy Current breast cancer Severe liver disease Undiagnosed vaginal bleeding History of hypersensitivity to medroxyprogesterone acetate or any excipients Severe cardiovascular disease Meningioma, current or previous
Cautions	Bone mineral density loss with prolonged use (review at 2 years) Weight gain Delayed return of fertility (median 5-6 months after last injection) Breast cancer history Migraine Diabetes Depression ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where the injection is given, in date, together with a telephone. A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering it must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training. The SmPC lists hypersensitivity reactions including anaphylaxis. Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint

	after an injection is common; anaphylaxis is very rare. Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain. OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER THE INJECTION, SEATED. Record that the observation period was completed. Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection. They are a response to the needle, not to the medicine. Have procedures in place to prevent injury from a faint. Users must be competent in intramuscular injection technique. Dispose of sharps in a UN-approved puncture-resistant container in accordance with HTM 07-01. Never re-sheath a needle.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Medroxyprogesterone acetate 150 milligrams per millilitre injection
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Deep intramuscular injection into gluteal or deltoid muscle
Dose and frequency	150 milligrams by deep intramuscular injection Repeat every 12 weeks (±5 days) First injection from 6 weeks postpartum if breastfeeding; from 21 days if not breastfeeding and no additional VTE risk factor (see inclusion criteria). Otherwise refer.
Quantity to be administered and/or supplied	Single injection per visit
Maximum or minimum treatment period	Every 12 weeks (±5 days) until change of contraception or pregnancy
Adverse effects	Very common: Menstrual irregularity, weight gain Common: Headache, dizziness, mood changes, acne, abdominal discomfort, decreased libido Long-term: Reduced bone mineral density (usually reversible)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • batch number and expiry date of the injection, and the injection site • that the 15 minute observation period was completed • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication. Discuss breastfeeding safety and the delayed return of fertility with Depo-Provera.
Follow-up advice to be given to patient or carer	Irregular bleeding is common with this method, particularly in the first few months. Fertility may take 5 to 6 months, and sometimes up to a year, to return after the last injection. This method is safe during breastfeeding. Return for the repeat injection every 12 weeks, no more than 5 days early or late. Seek medical advice if symptoms worsen or any adverse effects occur. Seek immediate medical attention if symptoms of DVT/PE develop

	(calf pain, swelling, breathlessness). Report any unexpected vaginal bleeding. Discuss contraceptive options if considering longer-term contraception.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- College of Sexual and Reproductive Healthcare (formerly FSRH), Contraception After Pregnancy, January 2017 (amended October 2020); UKMEC 2025; Summaries of Product Characteristics for the products supplied, current versions
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v008

Issued: 30 September 2026

Supersedes: Version 007, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v007

Issued: 14 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 42: the desogestrel arm names the UKMEC 2025 progestogen-only pill category 3 and 4 conditions as exclusions (current breast cancer; past breast cancer; ischaemic heart disease, stroke or TIA; severe cirrhosis; hepatocellular adenoma or carcinoma; enzyme-inducing medicines now or within 28 days; SLE with antiphospholipid antibodies), the inclusion line refers to that list, the caution for breast cancer more than 5 years ago is removed as contradicted, and the guidance summary states the categories. List drafted from UKMEC 2016 (amended 2019) as carried into UKMEC 2025, the UKMEC 2025 summary sheet could

not be fetched, for Chris Pilkington to confirm; SLE with antiphospholipid antibodies is included on the safer reading and its category should be confirmed.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Title corrected: supply of desogestrel and administration of Depo-Provera for postnatal contraception; it read for the administration of Postnatal Contraception for the treatment of Postnatal Contraception.

Depo-Provera arm now carries the intramuscular injection provisions written into the other injection documents on 10 September 2026: adrenaline immediately available, anaphylaxis and basic life support training, 15 minute seated observation, faint procedures, injection competence and sharps disposal. The redundant adolescent caution in an 18 and over arm is replaced by it.

Depo-Provera records now require the batch number, expiry date, injection site and completion of the observation period.

Follow-up advice written per arm; each arm carried the other arm's advice.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name CoSRH Contraception After Pregnancy and UKMEC 2025 as the governing sources. There is no NICE CKS topic on postnatal contraception.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body

of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Depo-Provera arm, exclusion added: meningioma, current or previous. The Depo-Provera SmPC (September 2025) contraindicates medroxyprogesterone in patients with meningioma or a history of meningioma; the previous version did not mention it.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

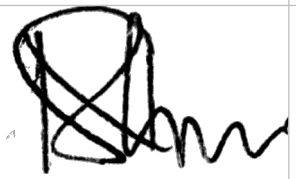
Changes in this version:

Desogestrel: initiation allowed at any time postpartum, as the document's own guidance summary and UKMEC 2025 (category 1 at every postpartum interval) say. The previous inclusion and dose row required day 21. Starts after day 21 need pregnancy excluded and 2 days of extra precautions (FSRH); the SmPC's 7 days is noted.

Depo-Provera: one early-start rule in place of three. From 6 weeks if breastfeeding; from 21 days if not breastfeeding and no additional VTE risk factor; otherwise refer. 'Per clinical judgement' is gone: it is not an auditable PGD criterion.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.